



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : صفاء ناشد خله .
 المحافظة : الجيزة الحي / المدينة : الجيزة
 رقم البطاقة : 26006032501121 رقم التليفون : 01224232656
 العمر (تاريخ الميلاد) : 03-06-1960 المهنة : ربه منزل
 العنوان : الجيزة
 تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-
 الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
 نفقات العلاج في حالة ثبوت غير ذلك
 الإخصائي الاجتماعي التوقيع: (-----)
 المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
 نتائج الأبحاث : (مرفق)
 التشخيص النهائي : سرطان بالثدي (مبادرة)
 خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
 التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
 الاسم : د/ندى حسن الوظيفة : التوقيع :
 الاسم : د/نرمين رمضان الوظيفة : التوقيع :
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
 بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس المجلس الطبي المتخصص مدير عام مديرية الشؤون الصحية ختم شعار الجمهورية

--

Name: صفاء ناشد خله National ID: 26006032501121	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
محافظة كفر الشيخ
شهادة الميلاد

صفاة
ناشد خلة
أبى محمد على الصيرى-ارض اللواء
العوزة- الجوزة

٢٦٠٠٦٠٣٢٥٠١١٢١

LM3227676

DM5

جنا/ موهى ف

٢٠٢٥/١١ ٢٦٠٠٦٠٣٢٥٠١١٢١

يكون على
التي
طاريف بختى صوفى
الطاقة سارية حتى ٢٠٢٢/١١/١٤

مترجمة
مستحقة

١

Patient Name : الأوردة / مينا نابل
Patient No : 26006032501121
Path No : 396/24
Sex : Female
Age : 63 Y
Referring Doc :

Request No : 13602090
Request Date : 28/01/2024
Verification Date : 12/02/2024
Financial Category : عيادات
Department : Out Patient
IHC No :
Registered by : Mrs Aya Moharrey

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 28/1/2024, 10:35 am.

Technique

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Results

- IPT (Positive control showed appropriate reactivity);
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 1% of tumor cells (score 4/8)
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 2+) For SISH
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II

IPT:

- ER: Positive (score 8/8)
- PR: Positive (score 4/8)
- Her2: Equivocal (score 2+)
- Ki67: Positive in 25% of tumor cells.
- For ISH (SISH).

Verified by :

DR Menna Nabil

Generated Date: 19/02/2024 12:00:23

Page 1 of 2

Printed By : Mrs Aya Moharrey

Patient Name الأستاذة منة نائل خليل
Patient No. 26006032601121
Path No. 390/24
Sex Female
Age 63 Y
Referring Doc.

Request No. 13002690
Request Date 28/01/2024
Verification Date 12/02/2024
Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by Mrs.Aya Mohamed

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :
DR.Menna Nabil

Printing Date : 12/02/2024 11:21:57

Page 2 of 2

Printed By : .marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt





Patient Name: Safaa Nashed Khellah	Date : 22/02/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 63 Year(s)
Medical ID: 591/2024	Pat. ID: 47085
	Total DLP (mGy.cm): 792

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast well defined mass lesion measured 4X3 cm.
- Few enlarged right axillary level I lymph node largest measured 4.2x3.2 cm
- Few bilateral scattered tiny pulmonary nodules largest in the left lower lobe measured 3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



Patient Name: Safaa Nashed Kheilah	Date : 22/02/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 63 Year(s)
Medical ID: 591/2024	Pat. ID: 47085
	Total DLP (mGy.cm): 792

- The scanned skeleton shows:
 - Spondylodegenerative changes.
 - Diffuse osteopenic texture with D12 structural collapse.
 - No definite worrisome lytic or sclerotic lesions.
 - L5/S1 degenerative 1st degree spondylolisthesis.

OPINION:

- *Right breast malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.*
- *Bilateral few pulmonary indeterminate nodules for follow up.*
- *No definite hepatic or osseous metastatic deposits.*

Dr. Elhadi Izaldeen

Dr. Beham Anwar, FEBR, MD



Patient Name: Safaa Nashed Khellah	Date : 21/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 63 Year(s)
Medical ID: 591/2024	Pat. ID: 47085

Combined bilateral breast mammography and complementary ultrasound

Indication:

RIGHT BREAST PATHOLOGICALLY PROVED CARCINOMA.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ spiculated irregular mass lesion, of high density reaching to nipple.
- Right mild retro areolar skin thickening.
- No left spiculated mass lesion or microcirculations.
- Complementary ultrasound revealed right breast 12 o'clock par areolar irregular spiculated hypoechoic mass lesion, with highly vascularity detected within on color Doppler, reaching to the nipple, measuring 55x30mm, consistent to the palpable lump.
- No left solid or cystic mass lesion.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Multiple Pathological right axillary lymph nodes are seen(less than 3 nodes) with absent hila, measuring 44x25mm.
- Nonspecific left axillary LN.
- Free supra clavicular lymph nodes.

Opinion:

- Right breast pathologically proved carcinoma with ipsilateral pathological axillary lymph nodes (less than 3 nodes) (BIRADS 6).
- left breast normal study (BIRADS 1)

Signature Dr. Nehal Hamdy

21/02/2024 1:18 PM



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : صفاء ناشد خله بخيت

Sex:

MDT form

3- Medical number:
Female

83189070

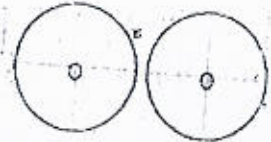
Old Med :

Old Med :

591/2024

2-ID number:

Patient Age : 65 Y



Family history

Family History

Phone

Phone

01272888022

BMI

BMI

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP

REASON OF MDT

REASON OF MDT
POST OPERATIVE

Co morbidity

Co Morbidity
free

Menopausal status

Menopausal Status

Initial STAGE

Initial STAGE

Date of MDT

Date of MDT

03/12/2024

Breast Imaging :

RT 12:00 55*30 mass
millpond prelegal LN

Clinical data

Mets Workup :

CT Bill pull indetennm index
nods for follow up

BX

IDC

BX COMMENT:

ER

8/8

ER COMMENT:

PR

4/8

PR COMMENT:

Her 2 Neu

2

Her 2 Neu COMMENT:

SISH

Not Amplified

SISH COMMENT:

Ki67

T

3

N

2

M

0

Sonomammography

PATIENT HISTORY

pt received NACT 3 EC+ 8 wks taxol last 14/8/2024 delayed multiple times bec of neutropenia
RT MRM : free margin
LN +ve
ypT3 N1

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

PORT

AI for 5-7 years

zometa every 6 months

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

د. يحيى اسماعيل

SURGERY

SURGERY

د. شريف جلال

Psychiatric

Psychiatric

RADIOLOGY

RADIOLOGY

د. محمد حسن

PATHOLOGY

PATHOLOGY

Nutrition

Nutrition

7777, 2777 Patient ID: 10110224005309 DOB: 30-Jul-2024 Date: 30-Jul-2024

Patient Lastname: Ali
Patient Firstname: Ali
RIS Patient ID: 356740
Date of Birth: 30/07/2024

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

99mTc MDP Bone Scan Report (30/07/2024)
* Whole Body Anterior and Posterior Planar Views Acquired 21hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" Non-uniform tracer uptake is seen over lumbar and lower dorsal vertebrae, likely marked degenerative changes for further CT correlation.
" No definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.
" Arthritic changes are noted over both shoulder, and knee joints.
" Both kidneys are visualized.
Conclusion:
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.
" Heterogeneous tracer uptake over lumbar and lower dorsal vertebrae likely pronounced degenerative changes for further radiological correlation.

Much Oblige
Mohamed Abdelhewab, MSc

REPORT TEXT ENDE

7777, 7777 Patient ID: 10110224005309 DOB: 30-Jul-2024 Date: 30-Jul-2024

Patient Lastname: Ali
Patient Firstname: Ali
RIS Patient ID: 356740
Date of Birth: 30/07/2024

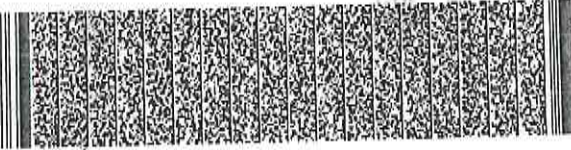
Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

REPORT TEXT
99mTc MDP Bone Scan Report (30/07/2024)
• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" Non-uniform tracer uptake is seen over lumbar and lower dorsal vertebrae, likely marked degenerative changes for further CT correlation.
" No definite evidence of active osseous deposits could be seen over the rest of the surveyed skeleton.
" Arthritic changes are noted over both shoulder, and knee joints.
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Conclusion:
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" Heterogeneous tracer uptake over lumbar and lower dorsal vertebrae likely pronounced degenerative changes for further radiological correlation.

Much Oblige
Mohamed Abdelhewab, MSc

REPORT TEXT ENDE



٢٠٢٥/١١ ٢٦.٠٦.٢٠٥.١١٢١

بدون عمل

مستبدلة

طريقا بجيت عوفى

٢٠٢٢/١١/١ : البطاقة سارية حتى

١




٢٦.٠٦.٢٠٥.١١٢١



صفاة

ناثا جده

٢ ش محمد على السبري-ارضى اللواء

المجوزة - الجيزة

بطاقة حقوق الانتخاب

LM3227676

Patient's Name: Safaa Nasheed Khela
Date: 2026-08-31
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST:

TECHNIQUE: Multislice axial CT images of the chest with multiplanar reconstruction. IV contrast was used.

FINDINGS:

- Evidence of right mastectomy.
- Left large pleural basal mass lesion measuring about 5.4x4.1
- Bilateral few pulmonary nodules, largest one seen at right lower lobe measuring about 14mm
- Minimal left pleural effusion
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No pericardial collection.
- No gross cardiomegaly
- Upper abdominal cuts are unremarkable.

Opinion:-

- Left large pleural basal mass lesion likely metastatic
- Bilateral few pulmonary nodules with suspicious right lower lobe for follow up
- Minimal left pleural effusion

Dr.Nourhan Ewis